

MEMORANDUM

TO: Health policy staff, House Energy and Commerce (Health)
FROM: ChemicalQDevice
DATE: June 5, 2026
RE: H. R. 9510, the Verification Before Generation in Physical AI Oncology Trials Act of 2026

Verify Robot Code Before It Touches a Patient (H. R. 9510)

10.5281/zenodo.xxxxxxxx

Independent research draft. Not an enacted law and not legal advice; not endorsed by the FDA, HHS, or any Member of Congress.

Subject

The problem

Cancer trials are placing robots in direct contact with patients, and what a robot does at the instant of contact is decided by software increasingly written by AI in real time. No Federal law governs the order of operations, so a sponsor may generate the code, run it, and document the checks afterward. Sequence matters because embodiment turns a caught-downstream error into a wrong motion in real time.

Why current law is not enough

The framework is substantial, but every piece stops short of requiring verification before generation.

1. The section 515C predetermined change control plan authority is the closest analogue, but it is voluntary, submission-based, and after-the-fact.
2. The Quality Management System Regulation (part 820) and part 11 records rules make a record trustworthy; they do not require it to come first.
3. The CMS Medicare Advantage guardrail (42 CFR § 422.101(c)) governs a coverage decision, not how robot code is cleared.

Figure placeholder (Images/sequencing-gap.png). The float, caption, and \label mirror the VVUQ-02 figures; replace the box with \includegraphics.

Figure 1: Figure 1. Placeholder schematic of the sequencing gap the bill closes.

The proposal

H. R. 9510 adds one new section, 515D, placed right after the section 515C change-control authority it resembles. The core rule is a single sequencing requirement bound to named external standards, with three hard catastrophe gates at perfect agreement.

Anticipated questions

Q. Does this create a new agency or program?

A. No. It adds one section to an existing statute and directs rulemaking only.

Q. Will it slow down cancer research?

A. The opposite; manual review cannot keep pace with machine-generated code.

Q. Does it preempt State law?

A. No. A savings clause preserves State human-in-the-loop requirements.

Recommendation

We recommend the Member consider introducing H. R. 9510 and refer it to Energy and Commerce. It closes a narrow, growing safety gap with a proven, low-cost mechanism that builds on tools the FDA and CMS already use.

Authority	What it does	Gap it leaves
§ 515C PCCP (§ 360e-4)	pre-authorize a planned change	voluntary; after-the-fact
Part 820 / part 11	quality and record integrity	no sequencing rule
42 CFR § 422.101(c)	human over AI in coverage	not trial code

Table 1: Table 1. In-force analogues at a glance (modeled on VVUQ-05 Table 1).

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Cite This Memo

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Data Availability

Supporting records are openly available in the [kevinkawchak/cancer-automated](#) and [kevinkawchak/physical-ai-oncology-trials](#) GitHub repositories and are archived on Zenodo. All data sets are synthetic or illustrative and reproducible from a deterministic seed; no patient-identifiable information is included.

References

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